



Accession No.	162648	Registration Date	: 02/04/2026 07:47:49
Patient ID	P16100023153	Sex / Age	: Male 41 Yrs 3 Mon 1 D
Patient Name	Mr. RAJEEV SAINI	Report Released on	: 02/04/2026 11:48:23
Client Name		Aadhar/ Passport No	:
Ref. By	Dr. Vineet Dubey		

DIGITAL WHOLE BODY PET CT

Clinical History: Case of growth nasopharynx. PET/CT study for further evaluation.

Procedure: 5.6 mCi of ^{18}F -fluorodeoxyglucose was administered intravenously. To allow for distribution and uptake of radiotracer, the patient was allowed to rest quietly for 60 minutes in a shielded room. Imaging was performed on an integrated 80-slice PET/CT scanner (UMI 550). CT images for attenuation correction and anatomic localization followed by PET images from vertex to mid-thigh were obtained. SUVmax was normalized to body weight SUVmax bw. Serum Creatinine and blood glucose was 0.75 mg/dL and 196 mg/dL respectively. CT scanning was performed using non-ionic intravenous (Volume: 50ml) and oral contrast & images were reconstructed to obtain transaxial coronal and sagittal views. Fused PET CT images were generated. No adverse reaction was observed during the scan.

Observations:

Brain: -

Normal physiological radiotracer distribution noted in the brain parenchyma. No focal lesion or abnormal FDG uptake noted in the brain.

(NOTE: If there is a strong suspicion for brain metastases / lesion, then MRI is suggested for further evaluation, as small lesions may not be detected on an FDG PET/CT study due to normal high physiological uptake in the brain).

Head and Neck: -

FDG avid soft tissue density mass noted involving posterior nasopharynx, measuring ~ 4.6 x 7.0 x 6.8 cm (AP x TR x CC), SUVmax: 29.4. Inferiorly, it is extending into the oropharynx, bilateral tonsillar fossa and base of tongue. Posteriorly, it is involving prevertebral space.

Hypopharynx and larynx appear unremarkable with no significant abnormal FDG uptake in relation to them.

Thyroid gland appears unremarkable with no focal abnormal FDG uptake.

FDG avid multiple discrete and conglomerate right level Ib, bilateral level II and left level III cervical lymphnodes noted, largest measuring ~ 4.6 x 3.0 cm, SUVmax 34.2. Other low grade FDG avid and non-avid bilateral level Ib, II to V cervical and supraclavicular lymphnodes noted.

Thorax: -

The heart and the mediastinal vascular structures are well opacified with I/V contrast. The trachea and main bronchi appear unremarkable.

Patchy ground glass opacity noted in bilateral lungs. Rest of the lung fields appear unremarkable. No focal abnormal FDG uptake is noted in the lung parenchyma.





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No obvious pleural thickening / effusion seen.

No significant enlarged FDG avid mediastinal and hilar lymph nodes.

FDG avid right level I axillary lymphnodes are noted, largest measuring ~ 1.6 x 1.3 cm, SUVmax 5.1.
Other non FDG avid bilateral axillary lymphnodes with preserved hilum.

Abdomen and Pelvis: -

Liver parenchyma is normal in attenuation values and enhancement pattern. No significant focal lesion / abnormal increased FDG uptake is seen. Intrahepatic biliary radicals are not dilated. Portal and hepatic veins appear unremarkable.

Gallbladder, pancreas, spleen, adrenals glands and bilateral kidneys appear unremarkable. (USG is the modality of choice to evaluate for cholelithiasis/choledocholithiasis).

No significant FDG avid abdominal lymphadenopathy noted.

There is no ascites.

The stomach, small and large bowel loops appear normal in calibre and fold pattern and show physiological FDG distribution.

The prostate appears unremarkable with no focal abnormal FDG uptake.

Musculoskeletal: -

Degenerative changes are seen in the spine.

No abnormal FDG uptake noted in axial and visualized appendicular skeleton.

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OPINION:

PET-CT study reveals: -

- Metabolically active soft tissue density mass involving posterior nasopharynx, as described above - likely primary malignant pathology.
- Metabolically active multiple discrete and conglomerated cervical and right axillary lymphnodes - likely metastatic.
- No other significant abnormal hypermetabolic lesion in rest of the body surveyed.

Clinical and HPE correlation is advised.

*This report is not valid for medico-legal purpose.
In case of any discrepancy due to machine error or typing error, please get it rectified.
Kindly bring all previous reports and PET- CT CD for follow up PET - CT scans.*

*** End of Report ***

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PATIENT NAME	: Mr. RAJEEV SAINI	AGE/ SEX	: 40 YRS / M
UID NO	: 882604040002	REGISTERED	: 04-Apr-2026 08:57AM
REFERRED BY	: Dr. RAJIV GANDHI CANCER INSTIT	COLLECTED	:
SAMPLE ID	: 8804002	REPORTED	: 05-Apr-2026 11:00AM

MRI

MRI NECK WITH CONTRAST

T1W AND FAST SPIN ECHO T2W HIGH RESOLUTION AXIAL IMAGES OF NECK WERE OBTAINED ON A HIGH RESOLUTION DEDICATED PHASED ARRAY SURFACE COIL AND CORRELATED WITH T2 WEIGHTED SAGITTAL, FLAIR AXIAL, AND FLASH AXIAL IMAGES. T1W POST-GADOLINIUM SCANS WERE TAKEN IN AXIAL, SAGITTAL AND CORONAL IMAGES AFTER IV ADMINISTRATION OF CONTRAST.

FINDINGS:

Significant diffuse circumferential wall thickening is seen of nasopharynx and oropharynx leading to the luminal narrowing. Thickening is ranging from 15-25mm.

Multiple enlarged bilateral level II and left level III lymphnodes are seen of sizes 10-30mm.

Post contrast study reveals mild enhancement of the thickening and lymphnodes.

Fossa of Rosenmuller on both sides appears normal. Bilateral laryngopharynx is normal.

Region of floor of mouth is normal. Tongue musculature (oral and base of tongue) appears normal. Retromolar region appears normal.

Bilateral valleculae and pyriform sinuses appear normal. Thyroid and cricoid cartilages appear normal. Region of thyroid, false and true vocal cords show no obvious abnormality.

Thyroid gland appears normal in size, shape and signal intensity.

Visualized submandibular and parotid salivary glands are normal.

Bilateral masticator spaces show normal signal intensity.

Bilateral parapharyngeal and carotid spaces appear normal.

Visualized trachea and esophagus appears normal.

Musculature of the neck appears normal.

IMPRESSION: MR imaging of neck reveals significant diffuse circumferential wall thickening of nasopharynx and oropharynx leading to the luminal narrowing. Multiple enlarged bilateral level II and left level III lymphnodes. Post contrast study reveals mild enhancement of the thickening and lymphnodes. Possibility suggested is ?Lymphoma. Advice: histopathological evaluation.

Dr. VARUN KUMAR
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CIN NO.: U74899DL1994PTC061085



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MRI

Please correlate clinically.

*** End of Report ***

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